

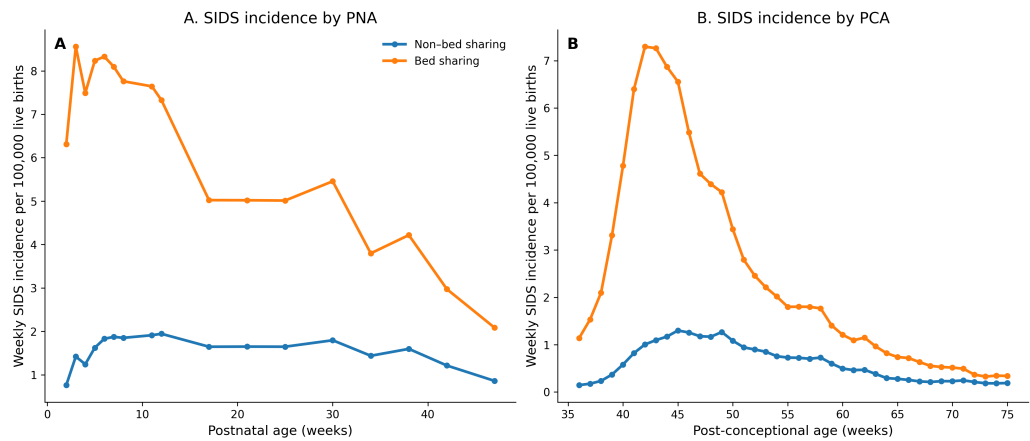


Figure 2. SUID incidence rates by sleep environment across chronological and developmental age scales.

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SUID incidence rates per 100,000 live births according to bed-sharing status, shown by (A) postnatal age and (B) postconceptional age (PCA). In panel A, rates are plotted on the chronological age scale and show highest incidence during the first several months of life, followed by decline. In panel B, rates are aligned by PCA, revealing a clearer developmental rise, peak, and decline structure. Bed-sharing and non-bed-sharing groups follow a similar overall developmental shape, but incidence remains consistently higher among bed-sharing cases across much of the trajectory.

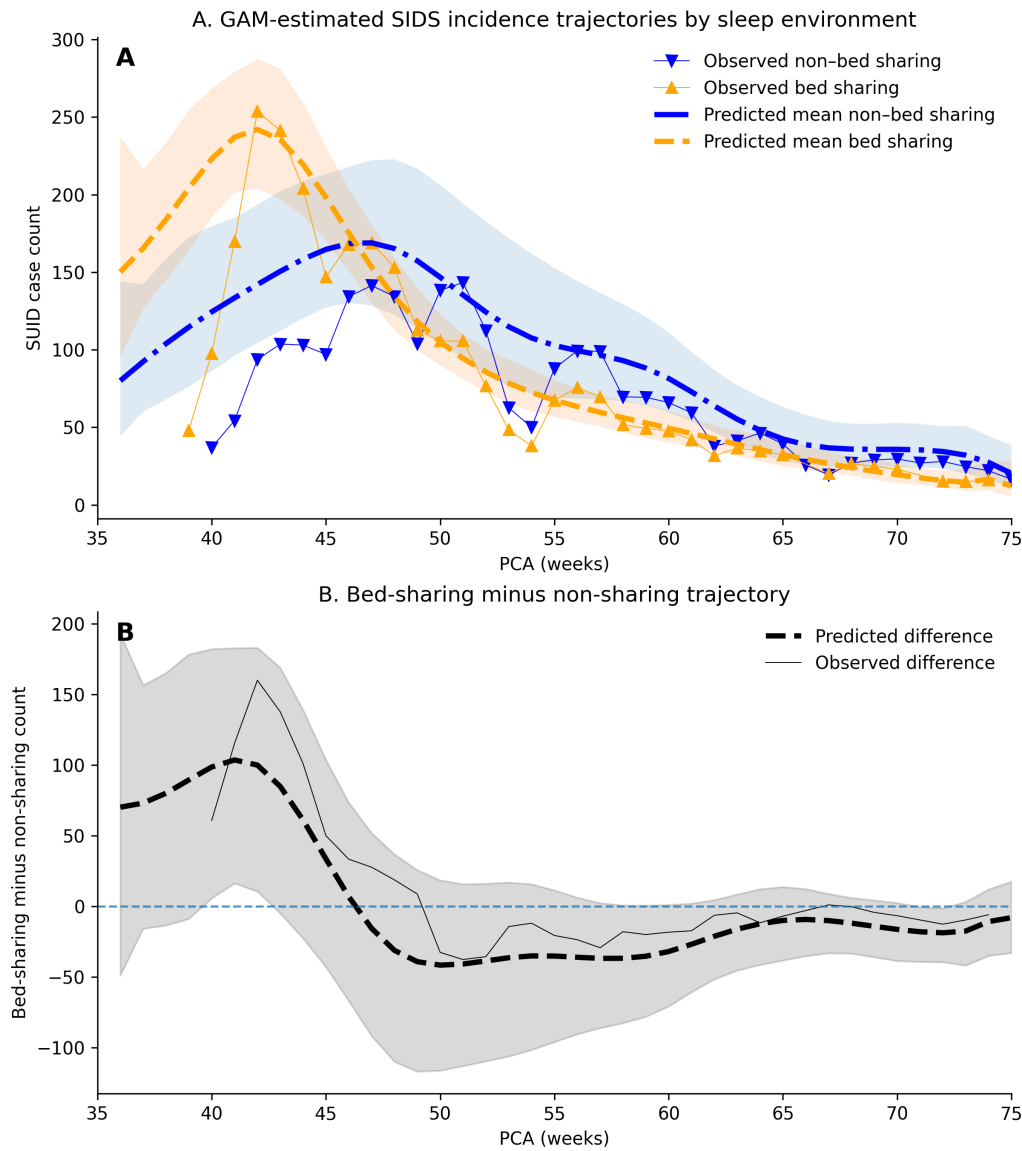


Figure 3A. Observed and predicted SUID case counts by sex and PCA.

Figure 3. Full-curve generalized additive model of developmental SUID incidence by sleep environment.

Observed and model-predicted weekly SUID case counts across postconceptional age (PCA) from a negative binomial generalized additive model, stratified by bed-sharing status. Points represent observed weekly counts; solid lines represent fitted mean trajectories; shaded bands indicate 95% confidence intervals. The fitted curves confirm a nonlinear developmental pattern characterized by an early rise, a peak in the mid-40-week PCA range, and a later decline. Across the curve, fitted incidence is higher among bed-sharing cases than among non-bed-sharing cases. The fitted trajectories further suggest that the bed-sharing curve peaks earlier and more sharply, whereas the non-bed-sharing curve shows a later, broader peak.

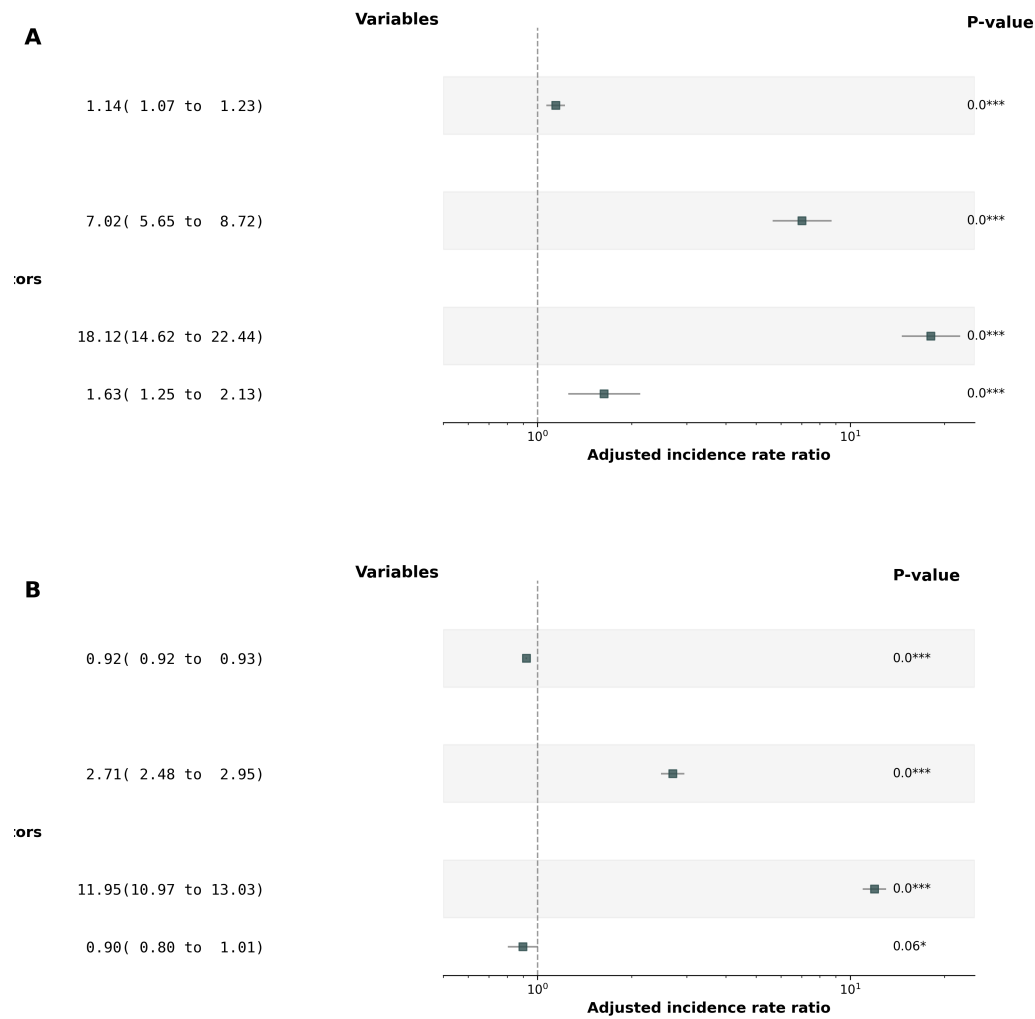


Figure 4. Observed and predicted SUID case counts by sex and PCA.

Figure 4. Adjusted incidence rate ratios from windowed negative binomial models across developmental phases.

Forest plot of adjusted incidence rate ratios (IRRs) and 95% confidence intervals from negative binomial models fit separately in the early rising and late declining developmental windows. Models included PCA in weeks, bed sharing, maternal smoking during pregnancy, and prematurity, with exposure-specific offsets based on external population denominators. In the early window, incidence increased with PCA and was independently associated with bed sharing, maternal smoking, and prematurity. In the late window, incidence declined with PCA, while bed sharing and maternal smoking remained strongly associated with higher incidence and the prematurity association attenuated.